TRIFLUOPERAZINE

High EPS
Less sedating

CHEMICAL NAME 10-[3-(4-methylpiperazin-1-yl)propyl]-2-(trifluoromethyl)-10H-phenothiazine (hydrochloride)

CLASS First-generation (typical) high-potency phenothiazine antipsychotic

TRADE NAMES Stelazine · Trifluoperazine (generic) · Parstelin (with tranylcypromine)

THERAPEUTIC USES / INDICATIONS

- Schizophrenia and other psychoses.
- Short-term severe anxiety (low dose, second-line).
- Severe nausea and vomiting.

ADMINISTRATION

- **PO** twice daily or once daily (MR).

DOSAGES

- **Psychosis:** 5 mg twice daily, increase by 5 mg after 1 week; usual 15–20 mg/day.
- **Anxiety/nausea:** 2–4 mg/day in divided doses (max 6 mg).
- **Elderly:** start at half dose.

PHARMACOKINETICS

- **Onset:** 2–3 h. Metabolised in liver.
- **t½:** 10–20 h.

NURSING MANAGEMENT

- Monitor EPS (AIMS scale), mood, BP.
- Anticholinergic for dystonia as ordered.

PHARMACODYNAMICS

- Potent **D2** antagonist; weaker anticholinergic and sedative effects than chlorpromazine.

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Extrapyramidal symptoms (frequent): akathisia, dystonia, parkinsonism.
- Dizziness, dry mouth.
- Hyperprolactinaemia.

ADVERSE EFFECTS (SERIOUS)

- Tardive dyskinesia; NMS.
- QT prolongation; blood dyscrasias.
- Cholestatic jaundice.

CONTRAINDICATIONS & CAUTIONS

- CNS depression; bone-marrow suppression.
- **Caution:** Parkinson's, epilepsy, cardiac disease, elderly.

KEY INTERACTIONS

- **Levodopa:** antagonism.
- **Alcohol, sedatives:** ↑ CNS depression.
- **QT drugs:** caution.

PATIENT EDUCATION

- Report restlessness, stiffness, tremor or abnormal movements.
- Avoid alcohol.
- Take regularly.

KEY POINTS

- High-potency phenothiazine.
- 5 mg BD start.
- Frequent EPS, less sedation.
- Tardive dyskinesia long-term.